



Name : Mrs. KEERTHANA M
Age / Gender : 32.10 Year(s)/ Female
Contact No. : +917604833651
Address : FLAT F PLOT NO 358 , 359 VEERALAKSHMI ...
Pin code : 600042

VID No. : 250217513123443
PID No. : P1572200062432
Referred by : DR.KOKILAVANI
Registered On : 10/01/2026 8:18 AM
Collected On : 10/01/2026 7:28AM
Reported On : 12/01/2026 4:18 PM

SUMMARY REPORT

Investigation Outside Reference Range (Abnormal)			
Investigation	Observed Value	Unit	Biological Reference Interval
Erythrocyte (RBC) Count (EDTA Whole Blood RBC Pulse Height Detection method)	4.16	mill/cu.mm	4.2-5.4
Haemoglobin (Hb) (EDTA Whole Blood Photometric measurement)	11.8	gm/dL	12.5-16
PCV (Packed Cell Volume) (EDTA Whole Blood Calculated)	35.5	%	37-47

Investigation Within Reference Range (Normal)
Blood Group (ABO & Rh Typing)
HBsAg Screening
HCV (Hepatitis C Virus) - Total antibody, Serum
HIV-DUO (IV th Generation test)
RPR

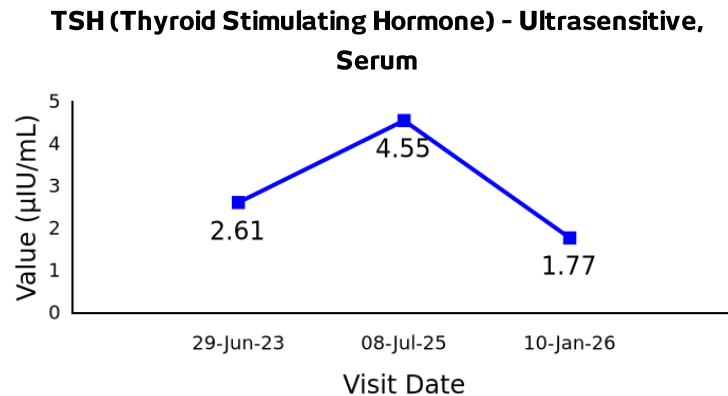
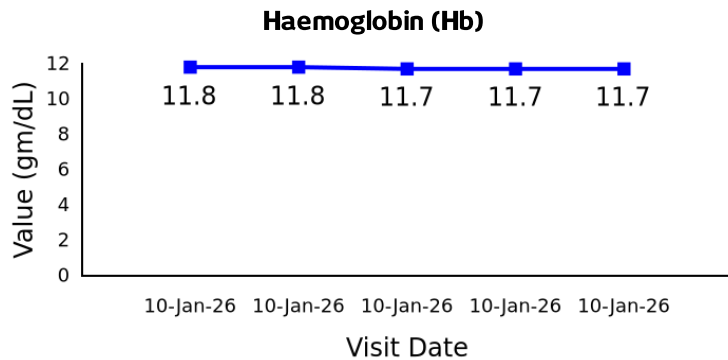
MEDICAL LABORATORY REPORT



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Result Trend (For selected tests used for followup)



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CBC Haemogram

Investigation	Observed Value	Unit	Biological Reference Interval
<u>Erythrocytes</u>			
Haemoglobin (Hb) (Photometric measurement)	11.7	gm/dL	12.5-16
Erythrocyte (RBC) Count (RBC Pulse Height Detection method)	4.00	mill/cu.mm	4.2-5.4
PCV (Packed Cell Volume) (Calculated)	34.1	%	37-47
MCV (Mean Corpuscular Volume) (Automated - Pulse Height Detection)	85.4	fL	78-100
MCH (Mean Corpuscular Hb) (Calculated)	29.1	pg	27-31
MCHC (Mean Corpuscular Hb Concn.) (Calculated)	34.1	g/dL	32-36
RDW (Red Cell Distribution Width)	13.9	%	11.5-14.0
<u>Leucocytes</u>			
Total Leucocytes (WBC) Count	8300	cells/cu.mm	4000-10500
Absolute Neutrophils Count	6474	cells/cu.mm	2000-7000
Absolute Lymphocyte Count	1328	cells/cu.mm	1000-3000
Absolute Monocyte Count	332	cells/cu.mm	200-1000
Absolute Eosinophil Count	83	cells/cu.mm	20-500
Absolute Basophil Count	83	cells/cu.mm	20-100
Neutrophils	78	%	40-80
Comments : Neutrophilia			
Lymphocytes	16	%	20-40
Monocytes	4	%	2.0-10
Eosinophils	1	%	1-6
Basophils	1	%	0-2
<u>Platelets</u>			
Platelet count	215	10 ³ /μL	150-450
MPV (Mean Platelet Volume)	9.2	fL	6-9.5
PCT (Platelet Haematocrit)	0.198	%	0.2-0.5
PDW (Platelet Distribution Width)	16.3	%	9-17

Danita Edwin

DR.DANITA EDWIN
MD (Pathology)
Consultant Pathologist
Reg No.96281

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Investigation

Observed Value

Unit

Biological Reference Interval

EDTA Whole Blood

- Tests done on Automated Five Part Cell Counter. (WBC,Platelet count by impedance method/DC detection,RBC by pulse height detection method, HB by Automated - Photometric Measurement, WBC differential by VCS technology other parameters calculated)

All Abnormal Haemograms are reviewed confirmed microscopically. Differential count is based on approximately 10,000 cells.

Danita Edwin

DR.DANITA EDWIN
MD (Pathology)
Consultant Pathologist
Reg No.96281

MEDICAL LABORATORY REPORT

Sample Collected At:

RA-MRS REKHA A (PALLIKARRI 2), FALT NO C.2 I TRANQUIL ACRES
PHASE 2 200 FEET MMRD ROAD, KANCHIPURAM CHENNAI...

Processing Location :

Processing LocationMetropolis Health care Ltd.#236,PR
Plaza,Velachery MainRoad,Selaiyur,chennai-600073



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Investigation	Observed Value	Unit	Biological Reference Interval
<u>Antenatal Profile - 1 (Maxi)</u>			
Glucose Fasting (Fluoride Plasma - F,Hexokinase)	85	mg/dL	Normal: 70-99 Impaired Tolerance: 100-125 Diabetes mellitus: > 125 (on more than one occassion) (American diabetes association guidelines 2018)

Note: An individual may show higher fasting glucose level in comparison to post prandial glucose level due to following reasons :
The glycaemic index and response to food consumed, Changes in body composition, Increased insulin response and sensitivity,
Alimentary hypoglycemia, Renal glycosuria, Effect of oral hypoglycaemics & Insulin treatment.

Associated Tests: HbA1c (H0018), Diabetes Profile – Maxi (D0021),HOMA Index (H0275), Insulin (I0275).

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Investigation	Observed Value	Unit	Biological Reference Interval
ESR (Erythrocyte Sedimentation Rate)	25	mm/hr	0-20

(EDTA Whole Blood,Westergren method)

Method: Automated Westergren

Interpretation:

- It indicates presence and intensity of an inflammatory process, never diagnostic of a specific disease. Changes are more significant than a single abnormal test.
- It is a prognostic test and used to monitor the course or response to treatment of diseases like tuberculosis, bacterial endocarditis, acute rheumatic fever, rheumatoid arthritis, SLE, Hodgkins disease, temporal arteritis, polymyalgia rheumatica.
- It is also increased in pregnancy, multiple myeloma, menstruation, and hypothyroidism.

Remark : ESR Performed using capillary photometric aggregation (for automated analysis) & westergrens (for manual testing).

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Investigation	Observed Value	Unit	Biological Reference Interval
TSH (Thyroid Stimulating Hormone) - Ultrasonic, Serum	1.77	μIU/mL	0.54-5.3

(Serum,Electrochemiluminescence immunoassay
(ECLIA))

Interpretation :

- Increased TSH is seen with intake of Iodine, Lithium, Amiodarone drugs and also indicates considerable physiologic & seasonal variation.
- Decreased TSH values require correlation with patient age & clinical symptoms and seen with intake of few drugs e.g. L-dopa, glucocorticoids.
- Transient alteration in TSH is seen in non-thyroidal illness like severe infections, liver disease, renal and heart failure, severe burns, trauma and surgery etc.

Clinical Utility: Levels of TSH are used for monitoring of thyroid related disorders.

Caution: Patients on Biotin supplement may have interference in some immunoassays. For sample collection, at least 8-hours wait time is recommended for individuals taking high dose of Biotin (more than 5 mg per day) supplements.

Note: TSH levels may fluctuate based on few factors such as pregnancy, illness and age. Also, time of sample collection, technologies used to analyze the test, usage of certain drugs, diet may have impact on TSH levels. TSH may show around 50% variation even when done at different times of day due to its association with circadian rhythm.

Associated Tests: T3 (T0029), T4 (T0031) free T3 (T0028), free T4 (T0030), reverse T3 (R1004), Thyroid Antibodies (T0061), Thyroid Comprehensive Profile-1 (T0062)

Reference:

- Clinical Chemistry 50:12, 2338-2344 (2004) and Ind J Clin Biochem (Apr-June 2014) 29(2):189-195.
- Ref: Arch Pathol Lab Med—Vol 141, November 2017.
- Fisher DA. Physiological variations in thyroid hormones: physiological and pathophysiological considerations. Clin Chem. 1996 Jan;42(1):135-9. PMID: 8565215

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Routine Examination Profile - Urine

Investigation	Observed Value	Unit	Biological Reference Interval
<u>Antenatal Profile - 1 (Maxi)</u>			
<u>Routine Examination Profile - Urine</u>			
<u>General Examination</u>			
Colour (Naked Eye Examination)	Pale Yellow		Pale Yellow
Transparency (Appearance) (Naked Eye Examination)	Clear		Clear
Reaction (pH) (Automated Photoelectric colorimetry)	7.5		4.5-8
Specific gravity (Automated Photoelectric colorimetry)	1.010		1.010-1.030
<u>Chemical Examination</u>			
Urine Protein (Albumin) (Sulpho salicylic acid method)	Absent		Absent
Urine Ketones (Acetone) (Rothera's method)	Absent		Absent
Urine Glucose (sugar) (Benedicts Test)	Absent		Absent
Bile pigments (Fouchet's method)	Absent		Absent
Bile salts (Hay's method)	Absent		Absent
Urobilinogen (Ehrlich method)	Normal		Normal
Nitrite (Automated Photoelectric colorimetry)	Negative		Negative
<u>Microscopic Examination</u>			
Red blood cells	Absent	/hpf	Absent
Pus cells (WBCs)	2 - 3	/hpf	0-5
Epithelial cells	1 - 3	/hpf	0-4
Crystals	Absent		Absent
Cast	Absent		Absent

Dr. R.LAKSHMI PRIYA
MD (Microbiology)
Doctor in Charge Microbiology
Reg No.87980

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Investigation	Observed Value	Unit	Biological Reference Interval
Amorphous deposits	Absent		Absent
Bacteria	Absent		Absent
Trichomonas Vaginalis	Absent		Absent
Yeast cells	Absent		Absent

Note : 1.

Chemical examination through Dipstick includes test methods as Protein (Protein Error Principle), Glucose (Glucose oxidase-Peroxidase), Ketone (Legals Test), Bilirubin (Azo- Diazo reaction),Urobilinogen (Diazonium ion Reaction) Nitrite (Griess Method).All abnormal results of chemical examination are confirmed by manual methods.

2.

Pre-test conditions to be observed while submitting the sample- First void, mid-stream urine, collected in a clean, dry, sterile container is recommended for routine urine analysis, avoid contamination with any discharge from vaginal, urethra, perineum,as applicable, avoid prolonged transit time & undue exposure to sunlight.

3.

During interpretation, points to be considered are Negative nitrite test does not exclude the urinary tract infections, Trace proteinuria can be seen with many physiological conditions like prolonged recumbency, exercise, high protein diet. False positive reactions for bile pigments, proteins, glucose and nitrites can be caused by peroxidase like activity by disinfectants, therapeutic dyes, ascorbic acid and certain drugs

.4.

All urine samples are checked for adequacy and suitability before examination.

Dr. R.LAKSHMI PRIYA
MD (Microbiology)
Doctor in Charge Microbiology
Reg No.87980

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Hemoglobin Variant Studies (Hb Electrophoresis)

(EDTA Whole Blood)

Investigation	Observed Value	Unit	Biological Reference Interval
Erythrocyte (RBC) Count (RBC Pulse Height Detection method)	4.16	mill/cu.mm	4.2-5.4
Haemoglobin (Hb) (Photometric measurement)	11.8	gm/dL	12.5-16
MCH (Mean Corpuscular Hb) (Calculated)	28.5	pg	27-31
MCHC (Mean Corpuscular Hb Conc.) (Calculated)	33.4	gm/dL	32-36
MCV (Mean Corpuscular Volume) (Automated - Pulse Height Detection)	85.3	fL	78-100
PCV (Packed Cell Volume) (Calculated)	35.5	%	37-47
RDW (Red Cell Distribution Width) (Derived from RBC Histogram)	13.6	%	11.5-14.0
Haemoglobin A0 (Hb A0)	97.4	%	94.3-98.5
Haemoglobin A2 (HbA2)	2.6	%	2.2-3.5
Impression	No evidence of haemoglobinopathy.		
Nucleated RBC (Automated Cell Counter)	-	per 100 WBCs	
Other	Normocytic, normochromic blood picture.		
Method:	Capillary electrophoresis - Whole blood		
Interpretations & Remarks:-			

Therese David V
DR.THERESE DAVID
DCP, DNB Pathology

MEDICAL LABORATORY REPORT



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1. All results have to be correlated with age and history of blood transfusion If there is history of blood transfusion in last 3 months, repeat testing after 3 months from last date of transfusion is recommended.
2. In case of haemoglobinopathy, parents or family studies,DNA Analysis and Genetic counseling is advised.
3. This test detects Beta thalassaemia and haemoglobinopathies. DNA analysis is recommended to rule out alpha thalassaemia and silent carriers.
4. Mild to moderate increase in fetal heamoglobin can be seen in some acquired conditions like Pregnancy, Megaloblastic anaemia, Thyrotoxicosis, Hypoxia, Chronic kidney disease, Recovering marrow, MDS, Aplastic anaemia, PNH, Medications (Hydroxyurea, Erythropoietin) etc.
5. Iron Deficiency Anaemia may be associated with low HbA2 result.
6. Megaloblastic Anaemia may be associated with elevated HbA2 levels (False high result).

Disclaimer: Report relate only to the sample as received. Refer to conditions of reporting overleaf.

Therese David V
DR.THERESE DAVID
DCP, DNB Pathology

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METROPOLIS HEALTHCARE LIMITED

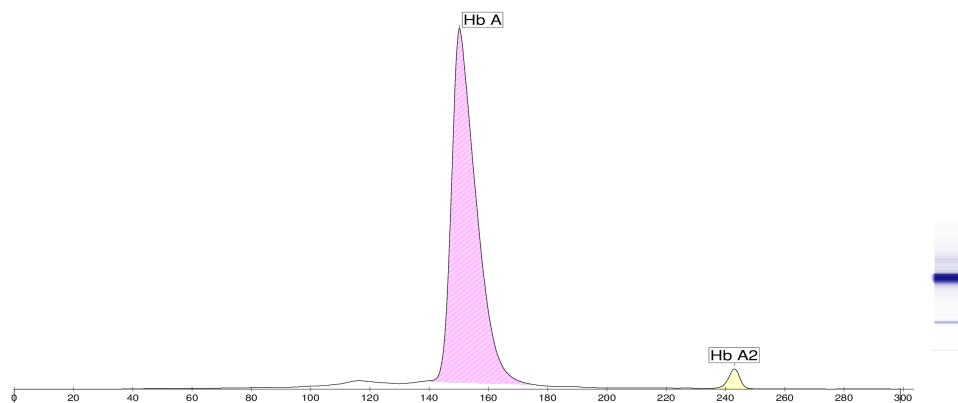
Name: KEERTHANA M
Age : 33

Sample ID : 0556108834
Report Date : 10/01/2026

Sex : F
Method:Capillary Electrophoresis

Hemoglobin Electrophoresis

Name	%
Hb A	97.4
Hb A2	2.6



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Investigation

Observed Value

Unit

Biological Reference Interval



Antenatal Profile - 1 (Maxi)

Blood Group (ABO & Rh Typing)

(EDTA Whole Blood)

Blood group (ABO typing)

B

RhD factor (Rh typing)

Positive

Method: Column agglutination technology (CAT) is an automated. System for blood grouping which is superior in sensitivity to conventional tube method.

Dr. SHANMUGAM
MD (Pathology)
Consultant Pathologist
Reg No.62032

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Processing Location :

Processing Location Metropolis Healthcare Ltd, #3, Jagannathan
Road, Nungambakkam, Chennai - 600 034



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Biological Reference Interval

Antenatal Profile - 1 (Maxi)



RPR

(Serum,Flocculation method)

Non Reactive

Non Reactive

Diagnostic titre - 1:8

Interpretation :

Comparison of RPR, TPHA tests

Test	Use	Limitations
RPR	Screening test. Follow-up titres for monitoring response to treatment.	Biological false positives possible. Repeat testing should be done with a specific test like TPHA for low titres such as 1:1, 1:2 and 1:4.
TPHA	More specific than RPR	Less sensitive in early syphilis.

RPR : Rising titres are found in active disease and levels subside after successful treatment.



HIV-DUO (IV th Generation test)

(Serum,Chemiluminescent Microparticle Immunoassay (CMIA))

Non Reactive(0.12)

S/CO

Non Reactive: < 1

Reactive: >= 1

Test Description : The HIV Ag/Ab Combo(HIV DUO) assay is a Screening test for simultaneous qualitative detection of HIV p24 antigen and antibodies to HIV type 1 and/or 2 in human serum or plasma. However, the HIV Ag/Ab Combo result does not distinguish between the detection of HIV p24 antigen, HIV-1 antibody, or HIV-2 antibody separately.

Test Interpretation :

- Non Reactive results may not rule-out acute or early HIV infection in the window period. If acute HIV-1 infection is suspected, detection of HIV-1 RNA or HIV proviral is recommended.
- Reactive results suggest possibility of preliminary infection with HIV-1 and/or HIV-2. All reactive samples should be verified by submitting a second serum specimen for repeat testing with screening & supplemental or confirmatory HIV tests (by serology-HIV 1 & 2 western blot or molecular- HIV 1 & 2 PCR).
- For the received samples, it is presumed that patient counselling is done at referring centre or by referring physician.



HBsAg Screening

(Serum,Chemiluminescent Microparticle Immunoassay (CMIA))

Non Reactive(0.00)

IU/mL

Non Reactive: < 0.05

Reactive: >= 0.05

All Reactive results must be confirmed by Neutralizing confirmatory test or by HBV DNA detection assay.



HCV (Hepatitis C Virus) - Total antibody, Non Reactive(0.08)

Serum

(Serum,Chemiluminescent Microparticle Immunoassay (CMIA))

S/CO

Non Reactive: < 1.0

Reactive: >= 1

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Interpretation :

This is a screening test detecting anti-HCV antibody. The CDC recommendation on anti-HCV testing includes the use of method specific optimal signal-to-cut-off ratio in interpretation & reporting positive results. For CMIA- s/co ratio - between 1 to 5- further supplemental tests are suggested for confirmation, while s/co ratio > or = 5 associated with 95% or more high probability of being true positive. Supplemental testing includes more specific assays like immunoblot or HCV RNA PCR for all positive results for confirmation or to further differentiate past(resolved) and chronic infections.

-- End of Report --



Test Marked with NABL symbol are in the scope of accreditation

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